MAXMETRICS · BODY COMPOSITION

# What you are made of today

Sam Smith · Male · DOB 14 June 1974 · twelve week retest · 10 December 2026

ILLUSTRATIVE SAMPLE — invented figures and invented age/sex markers for layout review. Live reports will not show a percentile until a real dataset is loaded.

Measured on a multi-frequency body-composition scale. This is not a clinical DEXA scan. Same-time-of-day repeats are what make it useful.

WEIGHT

## 80.0 kg

BMI 25.2 · height 178 cm

Your bodyweight on the day. Useful for loading calculations, but it does not tell you what the weight is made of.

BODY FAT

## 22.4%

17.9 kg fat mass

LOWER FAT HIGHER FAT

**22.4% of bodyweight**

The share of your weight the scale estimates as fat. It is a trend tool. Lower is not automatically better if muscle is being lost.

MUSCLE MASS

## 36.8 kg

LOWER HIGHER

**61st percentile**

Estimated skeletal muscle. More muscle supports strength, blood-sugar control, and staying capable with age.

### How the weight splits

Fat 17.9 kg Muscle 36.8 kg Bone est. 3.1 kg

Muscle is the green bar — the tissue that produces force. Fat is the gold bar. Bone is a scale estimate only.

VISCERAL FAT INDEX

## 8

LOWER HIGHER

**Device index, not a scan**

A scale index of fat around the organs. Higher readings are a reason to keep training and nutrition honest — not a CT diagnosis.

BODY WATER

## 55.1%

Estimated body water. A sudden jump or drop often means hydration, not a real change in fat or muscle.

BONE (SCALE ESTIMATE)

## 3.1 kg

A scale estimate of bone mineral, not a DEXA bone-density T-score. Do not treat this as an osteoporosis result.

### What this means for you

Most of today's weight is lean tissue. Fat mass is in a workable range for a man in his fifties. Muscle is the lever we can still improve. The visceral index is something to watch, not panic about.

This is a standing scale estimate (bioelectrical impedance), not a hospital DEXA scan. Hydration, food, and time of day move the numbers. Use it to track change under the same conditions, not as a medical diagnosis of bone or fat distribution.

### Change since last scale reading

| | | |
|---|---|---|
| Weight | 81.4 kg ’80.0 kg | 1.4 kg |
| Body fat | 24.1% ’22.4% | 1.7 pts |
| Muscle mass | 35.6 kg ’36.8 kg | +1.2 kg ‘ |

MAXMETRICS · NEUROMUSCULAR PROFILE

# Where you sit today

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MAXIMAL FORCE

## 161.2 kg

LOWER HIGHER

**68th percentile**

How hard you can push with both legs. This is the strength behind standing up, stairs, and carrying shopping.

RAPID FORCE · 200 MS

## 102.4 kg

LOWER HIGHER

**55th percentile**

How quickly that force shows up. It matters if you trip, stumble, or need to catch your balance.

GLOBAL ISOMETRIC

## 178.4 kg

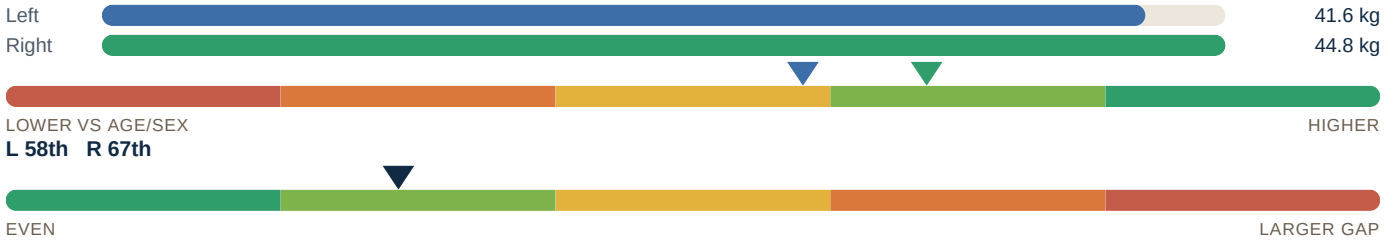
LOWER HIGHER

**72nd percentile**

This is your overall lower-body strength. It sits behind walking, hiking, standing from the floor, and getting through a long day on your feet.

## Grip strength

Measures maximal hand-grip force independently on each side. It is one of the simplest and strongest markers of overall capacity and longevity.




LOWER VS AGE/SEX HIGHER

**L 58th R 67th**



EVEN LARGER GAP

**7.1% difference · balanced**

Grip is how you hold shopping, open jars, and catch yourself on a rail. Lower grip for your age is a signal to keep building whole-body strength — it is not a hand injury diagnosis.

Best valid squeeze each side, not the average. Dominant hand: Right. Headline is best valid squeeze each side, not the three-trial mean.

Lower / larger gap · Middle · Higher / more even

## Regional symmetry at a glance

KNEE EXTENSION · LEFT LOWER

EVEN LARGER GAP

**7.0% difference · balanced**

This is front-of-thigh strength. You use it to stand from a chair, climb stairs, and squat to pick something up.

KNEE FLEXION · LEFT LOWER

EVEN LARGER GAP

**4.2% difference · balanced**

This is back-of-thigh strength. It helps you slow down, control a stride, and stay steady walking downhill.

HIP ABDUCTION · LEFT LOWER

EVEN LARGER GAP

**21.0% difference · priority**

This is side-hip strength. It helps you stand on one leg, get in and out of a car, and walk without the hip dropping.

UPPER-BODY PULL · LEFT LOWER

EVEN LARGER GAP

**5.4% difference · balanced**

This is pulling strength. You use it to carry bags, open a heavy door, and lift something toward you. Grip is tested separately.

## Change since 12 September 2026

This is how today's result compares with your last visit — often the most useful number on the page.

| | | |
|---|---|---|
| Knee Extension — Left | 42.8 kg ’48.9 kg | **+14.3%** ‘ |
| Knee Extension L:R asymmetry | 16.6% ’7.0% | **9.6 percentage points** “ |
| Grip — Right | 42.1 kg ’44.8 kg | **+6.4%** ‘ |

| | | |
|---|---|---|
| Ankle Mobility — Left | 7.8 cm ’8.2 cm | **+0.4 cm** |
| Ankle Mobility L:R difference | 3.1 cm ’2.5 cm | **-0.6 cm** |
| Shoulder Flexion — Left | 154° ’158° | **+4°** |

FORCE PLATE

# Maximal and rapid force

Sam Smith · Male · DOB 14 June 1974 · twelve week retest · 10 December 2026

ILLUSTRATIVE SAMPLE — invented figures and invented age/sex markers for layout review. Live reports will not show a percentile until a real dataset is loaded.

Measures maximal lower-body force, force relative to bodyweight, how quickly force appears, and how the two sides share the work.

PEAK FORCE

## 161.2 kg

LOWER HIGHER

**64th percentile**

The hardest you pushed today. Think standing up, stairs, and picking up a heavy bag.

RELATIVE PEAK FORCE

## 2.01 × BW

LOWER HIGHER

**68th percentile**

Force for your bodyweight. It answers whether that effort is strong for someone your size.

### Rapid force production

## 102.4 kg at 200 ms

LOWER HIGHER

**55th percentile**

How fast the force arrived. This is closer to catching yourself than to a slow, heavy lift.

### Left and right contribution

EVEN LARGER GAP

**12.6% difference · monitor**

Whether both legs shared the work. A difference is not automatically a problem, and it is not an injury diagnosis.

### Technical results

| | |
|---|---|
| Peak force (selected trial 2) | 161.20 kg |
| Force @100 / 200 / 300 ms | 78.4 / 102.4 / 128.6 kg |
| RFD peak / 100 / 200 / 300 | 2840 / 1960 / 1680 / 1420 N/s |
| Impulse peak / 100 / 200 / 300 | 412 / 38 / 96 / 168 |
| Time to peak (stored, not headline) | 412 ms |
| Mean force (stored, not headline) | 118.4 kg |

ISOMETRIC STRENGTH

# Regional force and symmetry

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Best valid trial, not the average. Blue arrow = left vs age/sex. Green arrow = right. The second bar is symmetry, which is a different question.

## Global Lower-Body Isometric

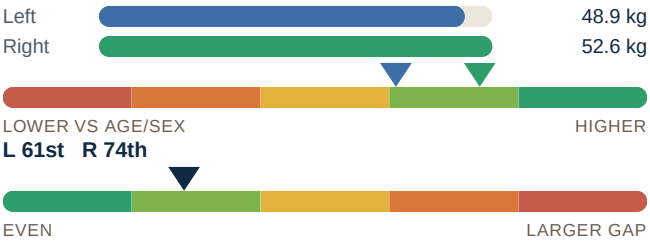
**178.4 kg**

LOWER HIGHER

**72nd percentile**

This is your overall lower-body strength. It sits behind walking, hiking, standing from the floor, and getting through a long day on your feet.

## Knee Extension

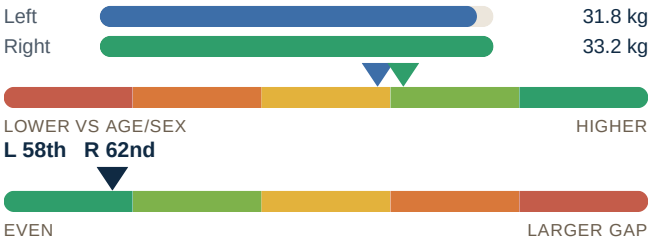



**L 61st R 74th**

**7.0% difference · balanced**

This is front-of-thigh strength. You use it to stand from a chair, climb stairs, and squat to pick something up.

## Knee Flexion

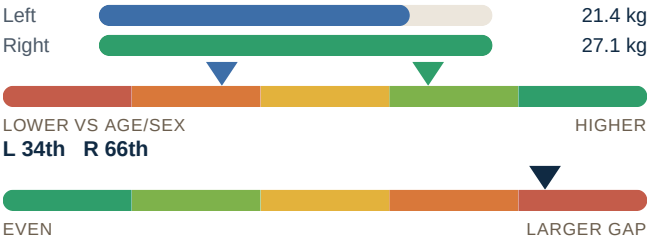



**L 58th R 62nd**

**4.2% difference · balanced**

This is back-of-thigh strength. It helps you slow down, control a stride, and stay steady walking downhill.

## Hip Abduction

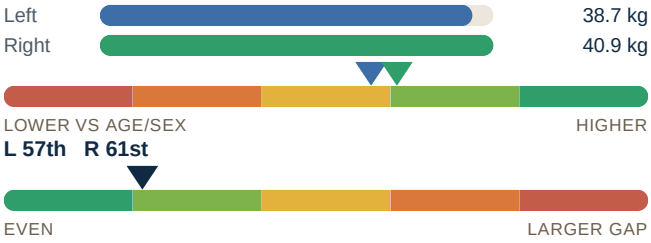



**L 34th R 66th**

**21.0% difference · priority**

This is side-hip strength. It helps you stand on one leg, get in and out of a car, and walk without the hip dropping.

## Upper-Body Pull




**L 57th R 61st**

**5.4% difference · balanced**

This is pulling strength. You use it to carry bags, open a heavy door, and lift something toward you. Grip is tested separately.

MAXMETRICS · MOBILITY & MOVEMENT

# How you move today

Sam Smith · Male · DOB 14 June 1974 · twelve week retest · 10 December 2026

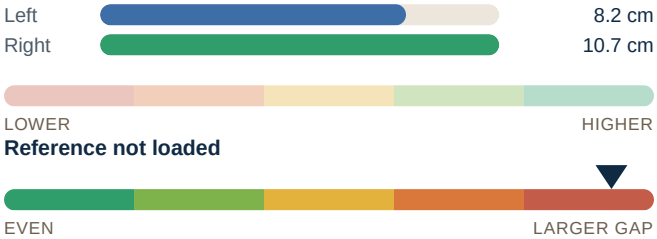
ILLUSTRATIVE SAMPLE — invented figures and invented age/sex markers for layout review. Live reports will not show a percentile until a real dataset is loaded.

These are measured ranges and observations, not a diagnosis of injury or pathology. Absolute mobility and left–right difference are shown separately. Age/sex markers appear only when a matching validated dataset is loaded.

## Mobility profile

| Test | Left | Right | Difference |
|---|---|---|---|
| Ankle Mobility | 8.2 cm | 10.7 cm | 2.5 cm · Left lower |
| Straight-Leg Raise | 72° | 81° | 9° · Left lower |
| Hip Internal Rot. | 31° | 39° | 8° · Left lower |
| Shoulder Flexion | 158° | 169° | 11° · Left lower |

## Ankle Mobility (Knee-to-Wall)




**Reference not loaded**

**23.4% difference · priority**

Ankle Mobility (Knee-to-Wall) range was lower on the left than the right (2.5 cm). No age/sex reference is shown because a matching validated dataset is not loaded. This is not a diagnosis of injury or dysfunction.

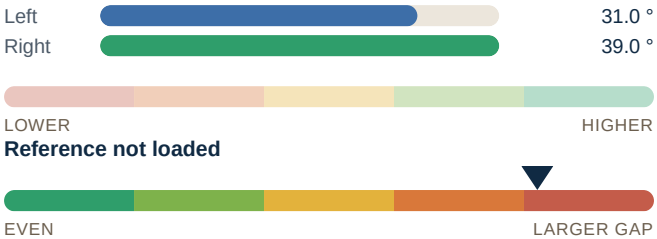
## Active Straight-Leg Raise

**Reference not loaded**

**11.1% difference · monitor**

Active Straight-Leg Raise range was lower on the left than the right (9°). No age/sex reference is shown because a matching validated dataset is not loaded. This is not a diagnosis of injury or dysfunction.

## Hip Internal Rotation

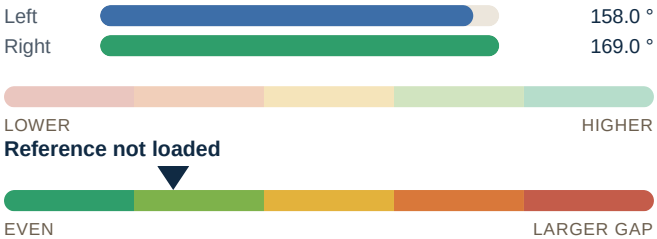



**Reference not loaded**

**20.5% difference · priority**

Hip Internal Rotation range was lower on the left than the right (8°). No age/sex reference is shown because a matching validated dataset is not loaded. This is not a diagnosis of injury or dysfunction.

## Shoulder Flexion




**Reference not loaded**

**6.5% difference · balanced**

Shoulder Flexion range was lower on the left than the right (11°). No age/sex reference is shown because a matching validated dataset is not loaded. This is not a diagnosis of injury or dysfunction.

## Overhead squat observation

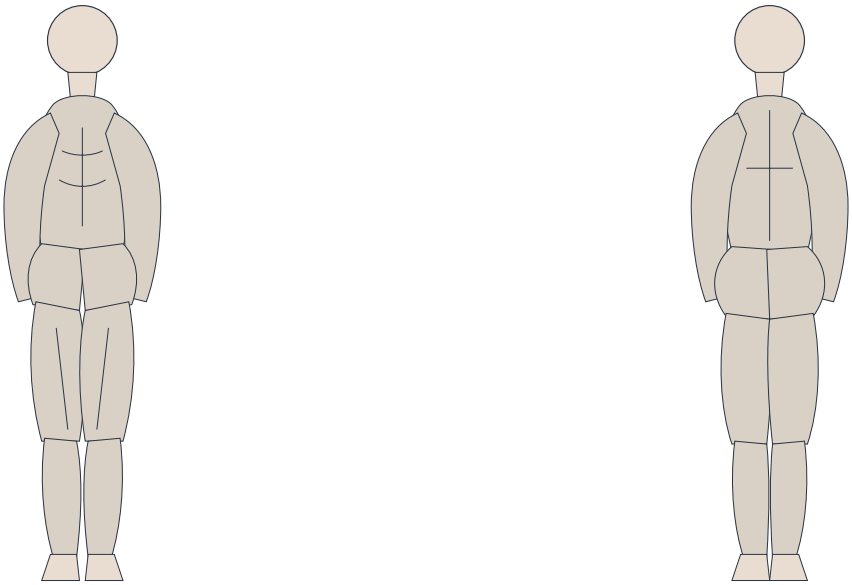
Depth PARALLEL · Heels BOTH_LOST · Shift NONE · Knees NO_OBVIOUS_ISSUE · Trunk FORWARD_LEAN_OBSERVED · Arms MAINTAINED · Pain NO

Both heels lifted during the overhead squat. Consider the ankle mobility results alongside this observation.

This observation is not a longevity score and has no population percentile.

## Mobility map

**Front** **Back**



Coloured only where a matching age/sex/protocol reference is loaded. Grey means not assessed or insufficient reference information.

Priority Favourable Grey = not tested / no matching reference

## Mobility findings for the next block

1. Ankle Mobility on the left may be considered a priority during the next training block. Keep the other side moving.
2. Monitor straight-leg raise on the left. A difference is present, but it is not automatically a problem.
3. Hip Internal Rot. on the left may be considered a priority during the next training block. Keep the other side moving.

WHAT THIS MEANS FOR YOU

# Map, findings, next 12 weeks

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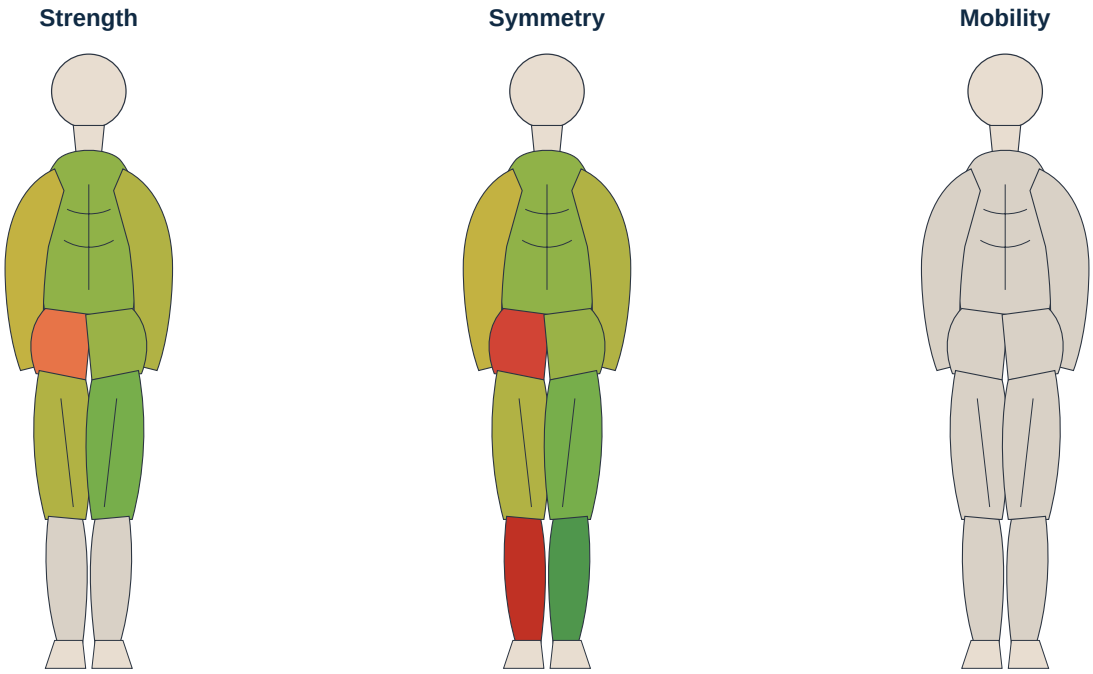
## Strength · symmetry · mobility

Three layers of the same figure. Strength uses loaded population comparison. Symmetry uses left–right difference. Mobility stays grey until a matching reference dataset is loaded — a lower side is not automatically painted red.




Priority — Favourable · Grey = not assessed / no matching reference

This diagram is an illustrative representation of assessment findings. Highlighted regions represent measured mobility and/or side-to-side differences and should not be interpreted as a diagnosis of injury, pathology or dysfunction.

## Key findings

1. Force-plate selected trial showed 12.6% peak-force asymmetry, with lower force on the left. This is a contribution finding, not a population-strength rating.
2. Knee Extension: 7.0% side-to-side difference (Balanced). Improved from the baseline test.
3. Hip Abduction: 21.0% side-to-side difference (Priority). This is the main regional symmetry priority for the next block.
4. Grip: left 41.6 kg, right 44.8 kg. A strong longevity marker, stored as best valid trial each side.
5. Ankle Mobility (Knee-to-Wall) range was lower on the left than the right (2.5 cm). No age/sex reference is shown because a matching validated dataset is not loaded. This is not a diagnosis of injury or dysfunction.
6. Both heels lifted during the overhead squat. Consider the ankle mobility results alongside this observation.

## Program priority · next 12 weeks

1. Consider prioritising appropriate unilateral work for the left side while monitoring symmetry over the next training block.
2. Maintain currently balanced loading and retest.
3. Maintain currently balanced loading and retest.
4. Maintain currently balanced loading and retest.
5. Maintain currently balanced loading and retest.
6. Ankle Mobility on the left may be considered a priority during the next training block. Keep the other side moving.